

Manuscript template (IMRaD)

Original research article — section by section

Foundations of Clinical Research · Sessions 7–8 (7.3, 8.1 Writing the paper) · Template version 1.0

How to use this template

- Replace every [grey italic text in brackets] with your own text. Use Ctrl + H → Find “[” to step through them, and check none remain before you share the document.
- Grey italic guidance and shaded “Example” boxes are there to help you. Delete them as you write: Home → Styles → right-click “Guidance” (or “Example”) → Select All → Delete.
- Write in this order: tables and figures → Methods → Results → Introduction → Discussion → Abstract → Title. Then reformat to your target journal’s instructions for authors (word limits, reference style, headings) — they always win over this template.
- Remove the Neudata header and this guidance before submission. Most journals want double line spacing and line numbers (Layout → Line Numbers → Continuous).
- **In the course:** each group uses this template for its draft manuscript from the FLUID-ICU course dataset — a STROBE-guided original article of about 2,000 words, using its results pack in Course/Data/. Methods and Results are drafted in Session 7, the rest in Session 8. See Assignment/manuscript_brief.md.

Reporting guideline reminder

- **CONSORT** — randomised trials (with the CONSORT flow diagram).
- **STROBE** — observational studies: cohort, case-control, cross-sectional.
- **STARD** — diagnostic accuracy studies.
- **PRISMA** — systematic reviews and meta-analyses (with the PRISMA flow diagram).
- Others (SPIRIT protocols, CARE case reports, SQUIRE quality improvement, TRIPOD+AI prediction models, SRQR/COREQ qualitative): find them all at www.equator-network.org. Fill in the checklist with page numbers and submit it with the paper.

Title page

Title:	<i>[Population, intervention/exposure, outcome and design: “...: a randomised controlled trial”]</i>
Running title:	<i>[≤ 50 characters]</i>
Authors:	<i>[First Last]¹, [First Last]² ... (ORCID iDs)</i>
Affiliations:	<i>[¹ Department, Institution, City, Country] [² Department, Institution]</i>
Corresponding author:	<i>[Name, postal address, email, phone, ORCID]</i>
Word counts:	<i>[Abstract n] · [Main text n] · [n tables, n figures]</i>
Article type:	<i>[Original research]</i>

Example: *Restrictive versus usual fluid strategy and 28-day mortality in adults with septic shock: a pilot randomised controlled trial.*

Abstract

Structured, usually ≤ 250 words. It must stand alone: the numbers in it must match the Results exactly. No spin: report the primary outcome first, with its effect size and 95% CI, even if it is not significant.

Background: *[Problem, gap, aim (2–3 sentences)]*

Methods: *[Design, setting, participants, intervention/exposure, primary outcome, analysis]*

Results: [*n analysed; primary outcome per group; effect size (95% CI); key secondary outcomes; harms*]

Conclusions: [*Answer the question, in words that match the design*]

Trial registration: [*Registry and number (trials)*]

Keywords: [*3–6 keywords, ideally MeSH terms, not repeating the title*]

Example: *Septic shock; Fluid therapy; Water-electrolyte balance; Intensive care units; Mortality*

Introduction

A funnel of three short paragraphs (about 400–500 words), from broad to your question.

Paragraph	What it does	Example sentence
1. What is known	The problem and its size; what is established (cited)	<i>Septic shock is common in ICUs and fluid is a core part of its treatment [1].</i>
2. What is unknown	The gap, conflict or limitation in the evidence	<i>Observational studies link a positive fluid balance to death [2], but cannot separate the effect of fluid from severity; trials are few and small [3].</i>
3. What we did	The aim or hypothesis, in one or two sentences	<i>We therefore conducted a pilot randomised trial to compare ...</i>

[Write the Introduction here]

Methods

Enough detail for another team to repeat the study. Past tense. Follow your reporting guideline item by item.

Study design and setting

Design, where, when (dates), registration number; protocol availability.

[Write here]

Participants

Eligibility criteria; how participants were identified and recruited.

[Write here]

Intervention / exposure and comparator

What exactly was done or measured, by whom, for how long.

[Write here]

Outcomes

Primary outcome (only one) and secondary outcomes, with definitions and time points.

[Write here]

Data collection

Sources, CRF, data management and quality checks.

[Write here]

Sample size

The calculation and all its assumptions.

[Write here]

Statistical analysis

Descriptive statistics; primary analysis with effect measure and 95% CI; adjustment; secondary and subgroup analyses; missing data; software; significance level. State that a SAP was written in advance.

[Write here]

Ethics

Committee name and approval number; consent procedure (or waiver).

[Write here]

Results

Participants FIRST: the flow diagram (screened → eligible → included → analysed, with reasons at each step) and Table 1. Then the primary outcome, then secondary outcomes and harms. Numbers with effect sizes and 95% CIs; no interpretation; each number in one place only (text OR table, refer to tables rather than repeating them).

Participants

[Flow (Figure 1) and baseline characteristics (Table 1)]

Primary outcome

[Result per group, effect size (95% CI), p]

Secondary outcomes

[Results (Table 2)]

Harms / adverse events

[Results]

Example: Of 412 patients screened, 60 were randomised and 58 analysed (Figure 1). By day 28, 9 of 29 (31%) patients in the restrictive group and 11 of 29 (38%) in the usual-care group had died (risk ratio 0.82, 95% CI 0.40 to 1.68). *[Fictional numbers, for illustration only.]*

Discussion

Six moves, in this order. Calibrate your language to the design: “was associated with” for observational studies; “caused” only for well-conducted trials; never “a trend” for $p = 0.42$.

Move	What to write
1. Answer	The main finding in one or two sentences — answer the question from the Introduction.
2. Compare	How it agrees or disagrees with previous studies (cited).
3. Explain	Possible explanations and mechanisms.
4. Limitations	Strengths and limitations, honestly: bias, confounding, precision, generalisability — and how each might change the result.
5. Implications	For practice and for future research (what study is needed next).
6. Conclude	A closing sentence that matches the design and the data.

[Write the Discussion here]

Conclusion

Two or three sentences. No new results; no claims beyond the data.

Example: In this pilot trial, a restrictive fluid strategy was feasible. Whether it reduces mortality needs a larger trial.

[Conclusion]

Declarations

Required by almost every journal. Check the journal's exact headings.

Declaration	Text
Ethics approval	[Ethics committee name], approval no. [number], dated [DD/MM/YYYY]
Consent	[Written informed consent / representative / deferred consent / waiver granted by ...]
Registration	[Registry, number, date (prospective / retrospective)]
Funding	[Funder, grant number, role of the funder — or “No specific funding”]
Competing interests	[“The authors declare no competing interests” or details (ICMJE form)]
Data availability	[Where and how the de-identified data and code can be obtained]
Use of AI	[Which AI tools were used, for what (e.g. language editing), and that the authors take responsibility for the content — or “No AI tools were used”]
Acknowledgements	[Contributors who do not meet authorship criteria]

Author contributions (CRediT)

Tick each role for each author (initials as column headings).

CRediT role	[AB]	[CD]	[EF]	[GH]	[IJ]
Conceptualization	☐	☐	☐	☐	☐
Data curation	☐	☐	☐	☐	☐
Formal analysis	☐	☐	☐	☐	☐
Funding acquisition	☐	☐	☐	☐	☐
Investigation	☐	☐	☐	☐	☐
Methodology	☐	☐	☐	☐	☐
Project administration	☐	☐	☐	☐	☐
Resources	☐	☐	☐	☐	☐
Software	☐	☐	☐	☐	☐
Supervision	☐	☐	☐	☐	☐
Validation	☐	☐	☐	☐	☐
Visualization	☐	☐	☐	☐	☐
Writing – original draft	☐	☐	☐	☐	☐
Writing – review & editing	☐	☐	☐	☐	☐

References

Use the journal’s style (often Vancouver: numbered in order of citation). Manage them with a reference manager. Cite original studies, not only reviews; check every reference against the source.

- 1. [Authors. Title. Journal. Year;Volume:Pages. doi]
- 2. [Authors. Title. Journal. Year;Volume:Pages. doi]
- 3. [Authors. Title. Journal. Year;Volume:Pages. doi]

Tables and figures

Each on its own page after the references (unless the journal says otherwise). Every table and figure must stand alone: a clear title, units, n per column, abbreviations explained in a footnote.

Table 1. Baseline characteristics of participants

Characteristic	Group A (n = __)	Group B (n = __)
[Characteristic, unit, summary]		
[Characteristic, unit, summary]		
[Characteristic, unit, summary]		

Data are n (%), mean (SD) or median (IQR). Abbreviations: ...

Table 2. Primary and secondary outcomes

Outcome	Group A	Group B	Effect (95% CI)	p
[Primary outcome]				
[Secondary outcome]				

Figure 1. Participant flow diagram

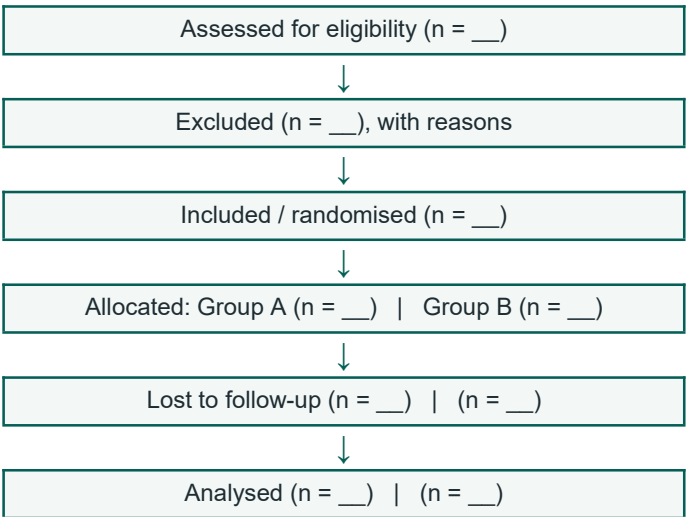


Figure legends: one per figure — title, what is shown, abbreviations.